PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	The effectiveness and safety of acupuncture therapy for inflammatory bowel disease: A protocol of systematic review and meta-analysis
AUTHORS	zhou, yuanfang; Sun, Ning; Cheng, Shirui; Deng, Xiaodong; Ye, Xiangyin; Li, Zhengjie; Zhou, Jun; guixing, xu; Qu, Yuzhu; Huang, Liuyang; Sun, Ruirui; Liang, Fan-rong

VERSION 1 – REVIEW

REVIEWER	Jun Zhao School of Acupuncture-Moxibustion and Tuina, Beijing University of Chinese Medicine
REVIEW RETURNED	30-Nov-2020

GENERAL COMMENTS	Please leave your comments for the authors below : 1. Although Crohn's disease (CD) and ulcerative colitis (UC) both have abdominal pain, diarrhea and rectal bleeding, their pathogenesis is completely different. There is great heterogeneity in the combined evaluation and analysis of the two diseases. Therefore, the subgroup analysis at the end of the protocol is less meaningful. It is suggested to narrow the topic of the article. 2. Acupoint catgut embedding is a special acupuncture method, which is widely used in inflammatory bowel disease (IBD). It should be clearly mentioned in the intervention method. 3. A good systematic review needs high-quality RCT research, but rare studies have been designed for IBD with acupuncture in recent years. 4. The outcomes are subjective indicators. It is recommended to add some objective indicators appropriately for more comprehensive evaluation.

REVIEWER	Liangjun Yang Tongde Hospital of Zhejiang Province
REVIEW RETURNED	30-Dec-2020

GENERAL COMMENTS	The methodological limitations of this protocol are as follows: 1. All the outcomes should be clearly defined. 2. The time of database searching is from inception to August 2020 or January 2020? 3. The methodology of risk of bias assessment is out of date. 4. The authors should explain how to interpret the results clearly, especially when SMD is used. 5. The authors' understanding of "unit of analysis issue" is incorrect. 6. The methods of subgroup analysis and sensitivity analysis should be explained clearly, not vaguely. 7. There are many grammatical and typing errors. To summarize, this protocol does not meet the expectation for the

	conduct and reporting of high-quality review and should not be accepted for publication.

VERSION 1 – AUTHOR RESPONSE

Reviewer #1:

Comment 1: Although Crohn's disease (CD) and ulcerative colitis (UC) both have abdominal pain, diarrhea and rectal bleeding, their pathogenesis is completely different. There is great heterogeneity in the combined evaluation and analysis of the two diseases. Therefore, the subgroup analysis at the end of the protocol is less meaningful. It is suggested to narrow the topic of the article.

Responses:

We really appreciate the advice given. However, although the CD and UC may have different pat hogenesis, they are two main subtypes of IBD with many common symptoms. Several RCTs on IBD[1-3]have tried to prove the effectiveness of the complementary and alternative therapy in improving those common symptoms regardless of the subtypes, and they did not distinguish the subtype during patient recruiting. As a results, if the topic is narrowed down to only CD or UC, these RCTs on acupuncture for IBD (both CD and UC) may be ignored with potential bias.

Therefore, without a prior hypothesis, the current meta-analysis will include RCTs on IBD, which may or may not distinguish different subtypes, and the subgr oup analysis will be carried out later to see if any difference is observed among different subtypes.

The reasons why we focus on the IBD instead of the CD and UC have been added accordingly. **(Page 6, Line 7-11)**

1 Horta D, Lira A, Sanchez-Lloansi M, et al. A Prospective Pilot Randomized Study: Electroacupuncture vs. Sham Procedure for the Treatment of Fatigue in Patients With Quiescent Inflammatory Bowel Disease. Inflamm Bowel Dis. 2020;26(3):484-492. doi:10.1093/ibd/izz091.

2 Berrill JW, Sadlier M, Hood K, Green JT. Mindfulness-based therapy for inflammatory bowel disease patients with functional abdominal symptoms or high perceived stress levels. J Crohns Colitis. 2014;8(9):945-955. doi:10.1016/j.crohns.2014.01.018

3 Schoultz M, Atherton I, Watson A. Mindfulness-based cognitive therapy for inflammatory bowel disease patients: findings from an exploratory pilot randomised controlled trial. Trials. 2015;16:379. Published 2015 Aug 25. doi:10.1186/s13063-015-0909-5

Comment 2: Acupoint catgut embedding is a special acupuncture method, which is widely used in inflammatory bowel disease (IBD). It should be clearly mentioned in the intervention method.

Responses: The acupoint catgut embedding has been supplemented in the "Type of intervention" (**Page 5, Line 18**) and **Table 1** #17.

Comment 3: A good systematic review needs high-quality RCT research, but rare studies have been designed for IBD with acupuncture in recent years.

Response: We have double checked the published RCT research and totally agree with the reviewer's advice. However, due to our plan for conducting a RCT of acupuncture on IBD, a systematic review on acupuncture for IBD treatment is really needed. In the future, we would conduct further systematic review on the complementary and alternative therapies for IBD so that more high-quality RCTs can be included.

Comment 4: The outcomes are subjective indicators. It is recommended to add some objective indicators appropriately for more comprehensive evaluation.

Response: The serum concentrations of α1- acid glycoprotein and C-reactive protein serum concentrations have been added as objective indicators for more comprehensive evaluation . **(Page 6, Line 23-25)**

Reviewer #2:

Comment 1: All the outcomes should be clearly defined.

Responses: The outcomes have been clearly defined and added in the outcomes. **(Page 6, Line 14-25)**

Comment 2: The time of database searching is from inception to August 2020 or January 2020?

Responses: In order to include more recent publications, the time is revised to 'from inception to March 2021. **(Page 7, Line 4)**

Comment 3: The methodology of risk of bias assessment is out of date.

Response: Thank you so much for your' suggestions. We have searched the published papers of meta-analysis again and found that the Cochrane Collaboration risk of bias tool is still commonly used even in those papers published with high quality.[1,2] Would you mind to tell us the better one so that we can better modify the methodology of risk of bias assessment ? **(Page 18, Line 27-30; Page 19, Line 1-6)**

1. He Y, Guo X, May BH, et al. Clinical Evidence for Association of Acupuncture and Acupressure With Improved Cancer Pain: A Systematic Review and Meta-Analysis. JAMA Oncol. 2020;6(2):271-278.

2. Mu J, Furlan AD, Lam WY, Hsu MY, Ning Z, Lao L. Acupuncture for chronic nonspecific low back pain. Cochrane Database Syst Rev. 2020;12:CD013814.

Comment 4: The authors should explain how to interpret the results clearly, especially when SMD is used.

Response: The interpretation of the results especially when SMD is used has been revised accordingly . **(Page 9, Line 24, 26; Page 10, Line 1-4)**

Comment 5: The authors' understanding of “unit of analysis issue” is incorrect.

Response: This part has been revised accordingly. **(Page 10, Line 20-21)**

Comment 6: The methods of subgroup analysis and sensitivity analysis should be explained clearly, not vaguely.

Response: The explanation of methods of subgroup analysis and sensitivity analysis have been revised accordingly. **(Page 10, Line 7-8)**

Comment 7: There are many grammatical and typing errors.

Response: The grammar and typing errors in the manuscript have fully been modified.